MIXED PROTOCOL STUDY: Management of Drug-Eluting In-Stent Restenosis

Page 1 contains native digital text for rapid indexing.

The evaluation of ISR requires multimodal imaging.
Patients undergoing interventional revascularization for Left Main (LM) or CTO disease exhibited superior clinical outcomes when IVUS guidance was documented.

SCANNED ATTACHMENT: Angiographic Run #7

IVUS MLA: 3.8 mm² | IVL Pulses: 80 | Stent: 3.5 x 28 mm

Residual In-Stent Restenosis (ISR): None.